



Interprofessional relationships and their impact on resident hospitalizations in nursing homes: A qualitative study

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ABSTRACT

Aim: The aim of this study is to explore experiences and perspectives of nurses and providers (e.g., physicians, medical directors, fellows, and nurse practitioners) on reducing preventable hospitalizations of nursing home (NH) residents in relation to interprofessional relationship and hospitalization decision-making process.

Background: Preventable NH resident hospitalization continues to be a pressing public health issue. Studies show that improved interprofessional relationship may help reduce hospitalization, yet research on communication processes and interactions among different NH staff remains limited.

Methods: This is a qualitative descriptive study. Two focus groups were held with fourteen nurses and thirteen in-depth, qualitative interviews were conducted with providers from two Chicagoland NHs. Focus group sessions and interviews were transcribed, coded, and analyzed for common themes based on qualitative description method.

Results: All study participants agreed that providers have the ultimate responsibility for hospitalization decisions. However, nurses believed they could influence those decisions, depending on provider characteristics, trust, and resident conditions. Nurses and providers differed in the way they experienced and conveyed emotions, and differed in key elements affecting hospitalization decisions such as structural or environmental factors (e.g., lacking staff and equipment at the facility, poor communication between the NH and hospitals) and interpersonal factors (e.g., characteristics of effective nurses or providers and the effective interactions between them).

Conclusions: Interpersonal factors, including perceived competence, respect, and trust, may influence NH hospitalization decisions and be targeted for reducing preventable hospitalizations of residents.

1. Introduction

Preventable resident hospitalizations in nursing homes (NH) represent a pressing public health issue in the USA. NH hospitalization rates range from 9 to 59 % (Grabowski et al., 2008; Spector et al., 2013; Xu et al., 2019), and 48.2 % of them are preventable (Grabowski et al.,

2008; Kayser-Jones et al., 1989). Resident populations have been rising over time, and as a result, NH nurses are facing higher workloads and struggling more in their interprofessional relationships (Fleischmann et al., 2016). Unlike many healthcare settings, NH nurses and providers (e.g., physicians, nurse practitioners) are not necessarily co-located when determining resident care. On-site nurses become off-site

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providers' "eyes and ears" as they discuss hospitalization decisions over the phone (Wagner et al., 2013). Providers rely on nurses' descriptions to determine whether to hospitalize residents. Such communication processes can be complex and may lead to poor NH resident outcomes, including preventable or unnecessary hospitalizations (Colón-Emeric et al., 2006; Madden et al., 2017). Although research has identified several factors associated with preventable hospitalizations (e.g. the availability of equipment and staffing on-site (Intrator et al., 2004; Lima et al., 2012; Thomas et al., 2013)), more work is needed to understand the role of interpersonal communication between nurses and providers.

Studies show that improved relationships among clinicians reduce sentinel events and potentially negative impacts on clinical outcomes (Martin et al., 2010; Tjia et al., 2009). For instance, Gittell et al. highlight the concept of relational coordination (RC) in patient care (Gittell et al., 2013), which refers to the network of communication and relationships among different workgroups involved in a common work process. Communication patterns are assessed by the frequency, timeliness, accuracy, and problem-solving nature of communication, whereas relationships are measured by the degree of shared goals, shared knowledge and mutual respect (Gittell, 2006). Similarly, a set of quality improvement programs called Interventions to Reduce Acute Care Transfers (INTERACT) were developed to improve the delivery of information between staff (Ouslander et al., 2014). These tactics include checklists to improve nurses' reporting of context and vitals. However, mixed outcomes have been reported on reducing hospitalization (Ingber et al., 2017; Kane et al., 2017; Ouslander et al., 2011). Moreover, other studies utilize qualitative descriptive research design to explore nurse-physician communication and collaboration (Boev et al., 2022). However, research on communication processes among different nursing home staff remains limited (Madden et al., 2017).

A nursing home in Illinois conducted lunch and learn sessions around communication to reduce unnecessary rehospitalizations. During the feedback session, we learned that although many of the nurses felt qualified to report on their patients, the results of their communication depended on other factors, including their relationship with the provider. As higher levels of RC are known to lead to better quality and efficiency performance (Gittell et al., 2013), this study aims to assess different features of interprofessional NH relationships and communication between nurses and providers to understand how they could impact unnecessary rehospitalization decisions. Through focus group sessions with nurses and interviews with providers, we explore their perspectives on hospitalization processes and the interprofessional factors that influence these decisions. Finally, we compare findings from both groups and offer insights about how to improve outcomes.

2. Methods

2.1. Design

We used a qualitative descriptive research design using focus groups and interviews.

2.2. Procedure

Nurses and providers were recruited from two NHs for focus groups and interviews, respectively, around the role of interprofessional interactions in NH hospitalizations. We recruited participants via emails, flyers, and word-of-mouth with the support of NHs. Nurses participated in two focus group sessions within NHs in June and December 2018. Providers were interviewed individually either in-person or via Zoom. Both NHs are operated by for-profit corporations, have 150–200 certified beds, and participate in Medicare and Medicaid (Fermazin et al., 2003). The NHs had identical staffing ratings, but differed in ratings for health inspections and quality measures (Fermazin et al., 2003).

A total of 14 nurses were recruited from the two sites (N = 9, N = 5). No physician assistants or nursing assistants were invited. Nurses were

78 % female. Thirteen providers, including Medical Directors (23 %), physicians (62 %), fellows (15 %) and nurse practitioners (15 %), were recruited—77 % were female. The UIC Institutional Review Board approved the study.

Participants were asked semi-structured questions regarding their perspectives on interprofessional relationships and NH resident hospitalization processes, including times when it was necessary and unnecessary. Questions about interpersonal interactions were driven by previous research on RC and the INTERACT models referenced above.

2.3. Analysis

Focus groups and interviews were recorded, transcribed and analyzed using qualitative description as described by Sandelowski, which produces findings that are "closer to the data as given, or data-near." (Sandelowski, 2010) This means that the interpretations of data tend to use participants' own words to describe the phenomena. It is particularly useful for providing "straight descriptions of phenomena" (Sandelowski, 2000) and in understanding the "who, what, and where of events or experiences" (Sandelowski, 2000) in areas where little may be known about the topic (Doyle et al., 2020; Sandelowski, 2010).

Three team members independently coded the first focus group transcript for emergent themes. Some emergent themes included differences in nurses' interpersonal relationships with providers, their use of emotions at work, and the non-routine aspects of their work (we expand on these below). Codes were defined and attached to quotes from the transcripts. A second group of three coders, including one from the initial coding team, used the identified codes to independently code the second transcript and updated the codebook with any new emergent themes. Disagreements were discussed until consensus was reached. Finally, both focus group transcripts were reviewed and re-coded with the updated codebook.

Three coders followed a similar procedure for provider interviews. Using the focus group code book as a starting point, they coded the first interview, redefining the codebook as new themes emerged. After reaching agreement on the first interview, they coded the rest, updating the codebook. The coders reached saturation by the seventh interview; no new themes emerged following Interview 7. Definitions of major themes can be seen in Fig. 1, whereas factors influencing preventable hospitalizations of NH residents derived from these themes can be found in Fig. 2. Comparison of nurse/provider responses, nurse responses, and provider responses are presented in Tables 1, 2, and 3.

3. Results

3.1. Theme 1: interpersonal relationships

Theme 1a: Nurses' interpersonal relationships with:

Providers. Nurses agreed that providers have the ultimate responsibility for hospitalization decisions and the nurse's role was to support those decisions. In clear cases (e.g., respiratory issues, irregular vitals, change in status), nurses hospitalized residents immediately, then notified providers. In less straightforward cases, nurses presented their observations.

In general, nurses felt influential in hospitalization decision-making. However, their perceived influence varied with each provider relationship. Some providers empowered nurses, asking their opinions. However, other providers did not or were likely to just send out residents regardless, minimizing nurse influence. Nurses felt providers' experience with lawsuits could have been an explanation for "most of these doctors that say 'send them out right away.'"

Nurses and Administrators. Nurses' relationships among themselves existed to obtain clarity about resident conditions. Before calling providers, they conferred with other nurses or the director of nursing (DON) to assess cases and decide how to influence

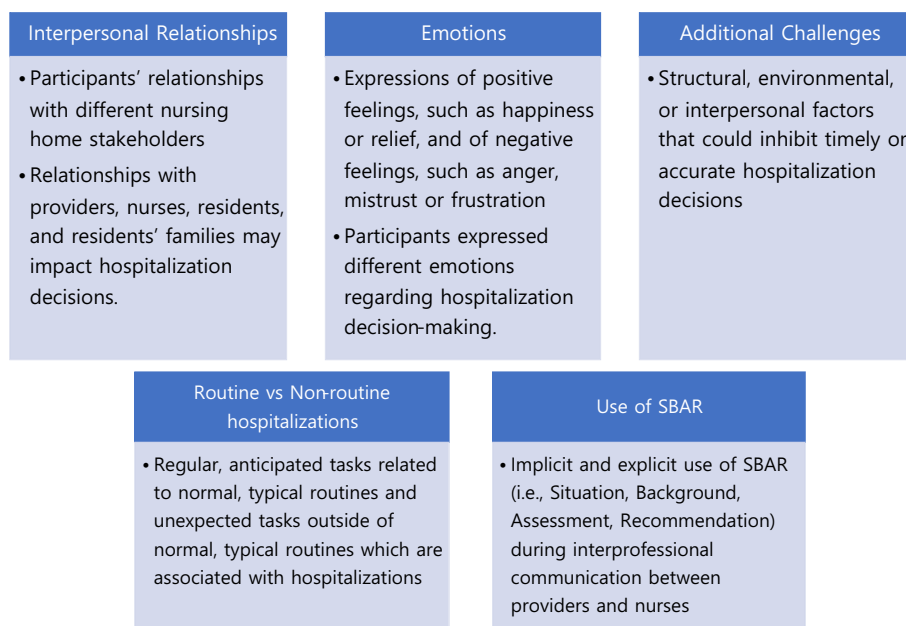


Fig. 1. Definitions of major themes.

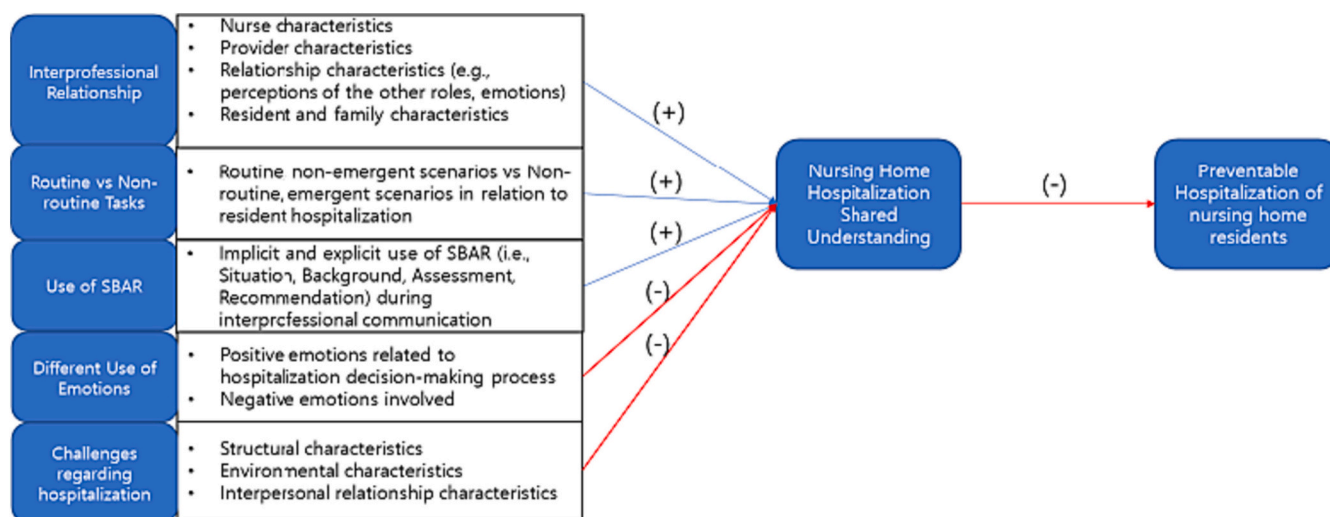


Fig. 2. Factors influencing preventable hospitalization of nursing home residents.

The positive arrows mean that when all things are aligned, (a) there is a stronger interprofessional relationship, which increases shared understanding; (b) during routine tasks, shared understanding increases; (c) when providers and nurses use SBAR, shared understanding increases. The negative arrows mean that different use of emotions and challenges regarding hospitalization leads to reduced shared understanding. Overall, increased shared understanding on nursing home hospitalization leads to reduced (or lower) preventable hospitalization of residents.

providers. For example, when nurses thought the resident has to go out but the “*doctor is not agreeable*,” they were likely to “*go to the DON or higher up*” for clarity and advice.

Residents and their Families. Residents and their families also influenced hospitalizations. Some nurses had sent residents to the hospital because of “*pressure from the family members*.” Residents also hospitalized themselves by calling 911, which was a situation that participants described as “*out of our hands*.”

Theme 1b: Providers' interpersonal relationships with:

Nurses. Providers had clear expectations of nurses: recognize acute symptoms, identify changes in resident status, and communicate case presentations effectively. These expectations shaped their perceptions of nurses' capabilities. Providers repeatedly mentioned their dependence on the nurses'

assessments. They also described gradually growing to trust some nurses, building the relationship and increasing the likelihood that they would listen to the nurse.

I knew who to trust and I knew who to not trust...if you always keep getting appropriate reports from the nurses and you agree with their assessment after you see the patient ... that builds up...a confidence... between you and the nurse.

Nurses who met or exceeded expectations were considered essential for on-site monitoring of complex patients who would otherwise need to be sent out for hospitalization. However, providers were less comfortable leaving residents in the care of nurses who didn't meet expectations, seemed ill-prepared to discuss the patient's

Table 1
Comparison of nurse and provider responses across key themes.

Theme	Role	Key Point	Quote
Interpersonal relationships	Nurse	Nurses pride themselves on being patient advocates and the providers' source of patient information.	<i>"It all depends on your observations 'cuz the doctors are not there. So you the doctor's eyes and ears."</i>
	Provider	Providers value their role as the key decision makers, and rely on their trust of the presenting nurse when evaluating evidence	<i>"Once you get that sense that the nurse did not really understand what's going on, then, then it's a problem."</i>
Emotions	Nurse	Nurses felt happy when providers listened to them, relieved when the providers made a tough call, and demoralized when the providers did not listen to them.	<i>"It's like they don't want to do anything. You know they don't want to do intervention first. It's like they don't trust us."</i>
	Provider	Providers tended to emphasize facts and their strong training and experience over emotions, but admitted that making important decisions remotely could be stressful.	<i>"You start thinking, you know, did I, did I do the right, eh, decision or you know, later on I'm gonna regret this"</i>
Additional challenges	Nurse	Nurses highlighted structural concerns, like lack of staff and equipment for unnecessary hospitalizations.	<i>"I think it's time and staffing, at the end of the day, because I can hear somebody coughing maybe but I have 39 patients to do."</i>
	Provider	Providers highlighted the difference in the purpose of nursing homes and hospitals, and complexity of resident cases for hospitalizations.	<i>"Things like that where you think, having lab work and imaging would help with decision making, you can send out to the emergency room."</i>
Interprofessional communication	Nurse	Nurses were familiar with SBAR, although they did not always reference it explicitly.	<i>"SBAR is really a good tool, like before you present your case to the doctor so you've gathered everything."</i>
	Provider	Providers were less familiar with SBAR. They emphasized vitals, physical exam, and detailed first-person knowledge of events.	<i>"Um, you know the, the present illness... And then the physical exam. Especially the, you know, vital signs"</i>
Routine vs non-routine hospitalizations	Nurse	Nurses emphasized variable staffing and patient health events as boundaries to routine.	<i>"There is no such thing as a typical day"</i>
	Provider	Providers desired stronger continuity of care between nursing homes and hospitals and nurse familiarity with common emergencies.	<i>"If you have a turnover in the nursing home, and you always getting new people, uh, going through a learning curve, uh, then it takes a while for them to understand how the system works."</i>

history and conditions, or with whom they did not have a prior relationship.

Residents and their Families. Providers described families as important resident advocates and valued their input in hospitalization discussions about the *"level of care interventions...they*

Table 2
Nurse responses across key themes.

Theme	Key point	Quote
Interpersonal relationships	Besides their own patient observations and residents' complaints, nurses described the impact of providers' different characteristics or experiences on hospitalization decisions. Additional influential entities included other nurses, Director of Nursing, residents, and families.	<i>"All the observations that we have on the patient and then we discuss it with the doctor. And then try to, cuz you're like the 'eyes and ears' for the doctor so you try to present [unintelligible]... And it's up to him. It's up to the doctor for the final decision."</i> <i>"Or if that nurse that, you know, who is familiar with the patient, and have been taking care of that patient for a few days, she might ah see the difference. She might know what's different with the patient."</i>
Emotions	Nurses felt a wide range of negative and positive emotions mainly based on the interactions they have with providers during hospitalization decision-making processes. When providers made decisions without knowing residents or ignored recommendations, nurses felt disrespected and frustrated whereas they felt relieved and supported when providers made the right call and listened to their recommendations.	Nurse-1: <i>"Mad, I mean like..."</i> Nurse-2: <i>"Frustrated."</i> Nurse-3: <i>"It's like they don't want to do anything. You know they don't want to do intervention first. It's like they don't trust us...The same thing like."</i>
Additional Challenges	Nurses first discussed structural and environmental factors like lack of time, staff, and equipment, and interpersonal factors such as relationship with providers that could hinder timely or accurate hospitalization decisions.	Nurse-1: <i>"I feel disrespected sometimes."</i> <i>"But some days you're going to be relieved cuz you're like, I know this patient needs to go get up out of here."</i> <i>"You feel good, yeah. You say, 'yeah the doctor understands.'"</i> <i>"Not having everything you need."</i> <i>"Sometimes you don't have the right equipment [unintelligible] ...you have to go look for stuff"</i> <i>"If the doctor is more receptive we are more comfortable telling everything that we saw and everything. And discussing it with him. If it's opposite, it's like you can't tell him really what's going on because they are, sometimes they are like, they are like rushing, rushing you on the phone."</i>
Use of SBAR	Nurses were familiar with the definition and usage of SBAR, particularly when there is a change of condition in residents, although sometimes they may use it implicitly.	<i>"SBAR, any new change to a resident, we usually initiate SBAR."</i>
Routine vs non-routine hospitalizations	Nurses discussed about expected tasks or tasks related to normal, typical routines, and about unexpected tasks or tasks outside the normal, typical routines that may lead to hospitalizations.	Routine tasks <i>"Medication and vital signs."</i> <i>"When the doctor comes in and gives us orders, it's what we do, carry it out."</i> Non-routine tasks

"Any respiratory issue we can't touch that."
"Airway, if we notice that they are short of breath. And we give them oxygen and still not go up. And if there's order for nebulization [...]"

(continued on next page)

Table 2 (continued)

Theme	Key point	Quote
		<i>and they are not improving and you see that they are really turning blue or all the stats are way down."</i> <i>"[...] or when they are screaming out in pain"</i>

Table 3
Provider responses across key themes.

Theme	Key point	Quote
Interpersonal relationships	Provider's evaluation of the quality of information from the presenting nurse is heavily dependent on past experiences with the nurse.	<i>...If the nurse has a good grasp of the situation and you think you can continue to work with the nurse overnight until the next day, then you can make a decision to keep the patient...in the nursing home.</i>
Emotions	Providers expressed a range of positive and negative emotions about making important decisions regarding hospitalizations remotely.	<i>I mean with a lot of the nursing home calls when you're not there I'm very much like, I wish I was there...so I can see the person so it's very tough trying to make the call over the phone...and then you just kind of send them out to be safe.</i>
Additional challenges	Providers highlighted how the professional environment, interpersonal relationships, liability concerns, resident characteristics and lack of continuity of care presented challenges in hospitalization decision making.	<i>Even the nursing home itself has the issues where they... it's not their fault but there is one nurse for the entire hallway so like ten patients or something.</i> <i>There could be somebody who... got sued and really got burned and then they're just like no send everybody in...like you're really nervous and scared...</i>
Use of SBAR	Although providers were less familiar with the SBAR tool compared to nurses, they listened for SBAR components (vital signs, physical exam findings, changes from baseline) from the nurses' presentation of residents.	<i>You know, I know there's a form I know nurses are supposed to use that form when they're calling a provider to explain the situation...you know, I probably have some type of SBAR in my brain that I... kind of use when I talk to a physician, but I don't follow an exact form</i>
Routine vs non-routine hospitalizations	Providers described hospitalizations as either needed or preventable depending on the reasons for hospitalization. Preventable hospitalizations included increased need for frequent monitoring or equipment.	<i>...And say I think this person needs to go out. And then...they end up getting admitted. And so it was a good thing that they ended up going out. And then on the flip side, I have seen it too where, you know, where we, I feel like the patient could be monitored just a little bit more closely.</i>

want.” Moreover, providers described the importance of aligning care with residents’ goals and preferences, even when they contrasted with the provider’s (or the family’s) beliefs.

3.2. Theme 2: emotions

Nurses. Nurses expressed a range of negative and positive emotions in hospitalization interactions. Some nurses described feeling frustrated and disrespected when providers ignored their recommendations. Some felt helpless and heartbroken when residents had Do Not Resuscitate (DNR) orders and when they could not “do something.”

Others described frustration when they felt providers made decisions without knowing residents and anger when residents were hospitalized for trivial issues and then immediately sent back to NHs. Unnecessary hospitalizations put residents at risk, and burdened nurses with paperwork.

Conversely, nurses described feeling relieved when providers’ decisions were right. They felt supported when providers followed their recommendations and provided positive feedback such as “Good catch.”

Providers. Like nurses, providers expressed negative (e.g., anxiety and frustration) and positive emotions. For instance, a provider may feel concerned when they felt that the “patient needed to be at a level of care that we were unable to provide” as the provider wants to “do what’s best for the patient quickly.”

In addition, the nature of NH care creates unique emotional challenges: wanting to “fix everything” yet having to “respect the patient’s wishes and [do] what they want,” which sometimes makes providers feel sad.

Despite these emotional challenges, most providers stated that their decisions led to positive outcomes, leading to increased comfort and confidence over time. Unlike nurses, providers tended to downplay the relevance of their emotions in interviews, focusing on facts that informed their decisions.

3.3. Theme 3: additional challenges

Nurses. Nurses described two key challenges in hospitalization decisions: structural/environmental factors and interpersonal interactions. Structural/environmental challenges included inadequate staffing levels, a lack of supplies (e.g., IV), and delays in getting medicine or lab results. Interpersonal challenges involved working with providers who were “stuck in their ways” and less receptive to recommendations. Nurses said that providers who rushed them on the phone or cut them off during case presentation reduced their comfort and participation.

Providers. Providers described similar challenges, including structural/environmental factors and interpersonal interactions, but also highlighted resident characteristics, impaired continuity of care, and liability considerations.

Providers’ structural/environmental challenges mirrored the nurses’, and included limited capacity, staffing, and equipment including slow access to labs, which delayed detection of disease. Every provider mentioned limitations in NHs, especially compared to ER or hospital environments. One participant described how “treatment in the nursing home is different from (my) treatment in the clinic.” They had ordered labs in the NH that simply disappeared.

Interpersonal challenges arose when there was a “big disconnect in the patient’s desires and the family’s desires,” or when providers’ expectations differed from NH administrators who didn’t want to send residents out due to high numbers of hospitalizations.

Providers noted that geriatric patients’ complex medical conditions, comorbidities, medication regimens, and symptom presentation meant that they were more challenging to diagnose. Poor continuity of care due to lack of communication between hospitals and the NH meant that ambulances rarely took residents to the hospital where they typically received care and providers rarely received adequate discharge reports from the hospital. Therefore, residents were more “likely to get sent out again, because there’s not a good discharge plan, and things get missed.”

Lastly, providers described the role of liability on their decisions. Providers sometimes send out the patient as “*liability might come into the picture*” if the patient isn’t sent out and something goes wrong later.

3.4. Theme 4: interprofessional communication

Nurses. Nurses mentioned SBAR (“Situation, Background, Assessment, Recommendation”) as a routine tool for assessing residents and communicating with providers. They initiated SBAR when there was “*any new change to a resident*,” and described it to be effective and “*really a good tool*” when presenting cases to doctors. Participants used SBAR explicitly or implicitly, where they followed the framework without spelling out each section.

Providers. Providers were less familiar with the use of SBAR. They listened for optimal case presentations from nurses, which implicitly followed the SBAR components—leading with a key concern and then including details about residents’ vital signs, exam findings, medications, and changes from baseline. Providers said they rarely heard recommendations from nurses, and perceived nurses to refrain from giving recommendations.

3.5. Theme 5: routine versus non-routine hospitalizations

Nurses. When asked about a typical day, one nurse replied, “*There is no such thing as a typical day*.” Changing resident conditions and staffing levels meant that routine tasks, including assessing residents’ conditions, providing medications, preparing for upcoming appointments, and following doctor’s orders, would sometimes shift to accommodate the idiosyncratic nature of care work. Recognizing emergency conditions, such as respiratory issues, was also routine. Less straightforward decisions occurred when residents seemed stable, only to have their condition decline shortly thereafter.

Providers described the nature of NH hospitalizations as emergency, unnecessary or preventable, and they defined ‘preventable’ as cases where residents needed more frequent monitoring than the NH could provide, or providers needed better information than provided by the nursing staff.

Providers agreed that increasing staff-to-resident ratios and continued education of the nursing staff “in making their own judgment assessment” would reduce preventable hospitalizations. Many providers focused on the lack of continuity of care for nursing home residents, either due to frequent staff turnover or lack of standardized protocol for patient hospitalization.

4. Discussion

In terms of relational coordination, communication patterns are assessed by the frequency, timeliness, accuracy, and problem-solving nature of communication, whereas relationships are measured by the degree of shared goals, shared knowledge, and mutual respect (Gittell, 2006). The results of our focus groups and interviews suggest that in addition to frequently researched structural or environmental factors (Ault et al., 2015; Grabowski et al., 2008; Intrator et al., 1999; Intrator et al., 2004; Lima et al., 2012; Thomas et al., 2013), interprofessional relationships in NHs could play a role in unnecessary resident hospitalizations. Consistent with research on relational coordination, we found that despite the remote nature of the work in nursing homes, nurses and providers share the same goals around providing high-quality patient care, allowing them to align many care-related activities. However, we found other domains where aspects of their roles may not align, emphasizing the importance of mutual respect and shared knowledge to improve shared decision making. Although they share some medical knowledge, nurses and providers in our study emphasized different types of knowledge and ways of communicating. For example, nurses felt a strong connection to the patient, indicating that they really

knew them (possibly as individuals with specific needs and preferences), whereas physicians wanted to leverage their knowledge of the patient’s condition to avoid missing a diagnosis or increasing liability. Use of tools like SBAR was mentioned as being helpful in providing timely and accurate communication, but some nurses stated they did not feel welcome to engage in the problem-solving process and some providers did not remember nurses using SBAR. Trust and mutual respect seemed to enhance the impact of such tools.

In addition, consistent with previous literature, we find that caregivers are not interchangeable – nurses and providers change how they interact based on whom they talk with. Since providers and nurses are not necessarily co-located, they rely on their relationships to determine whether to send residents to the hospital. Three inter-group factors influencing these relationships were roles, relationship quality, and emotions:

4.1. Roles

Participants were clear that providers’ roles included ultimate responsibility over hospitalizations and that nurses served as the “eyes and ears” of NHs. Nurses were patient advocates who knew the residents. Based on their knowledge of residents’ conditions, direct observations, and “gut instinct,” they watched for new onset of disease. Providers, on the other hand, were action-oriented and wanted to hear the necessary details of the problem so that action could be taken (Nadzam, 2009). They emphasized their role as problem solvers and described using their clinical experience to take the best action for residents based on nurse communication. Although there was clarity in these roles and their boundaries, there were conflicting incentives associated with each. Providers were incentivized to hospitalize residents when unsure about residents’ conditions or the NH’s ability to provide adequate care. Nurses were incentivized to protect patients from the stress of hospitalization and the ensuing paperwork, when they perceived that they could provide adequate care onsite.

4.2. Relationship quality trust/input

Nurses and providers described the quality of the relationships differently. Providers described three expectations of nurses: recognizing acute symptoms, identifying changes in resident status, and communicating case presentations effectively. Providers trusted nurses who met these expectations and felt comfortable leaving residents in their care. When providers did not trust nurses, they tended to hospitalize residents, where there were more resources to monitor and care for residents. Our findings resonated with another study which identified trusting relationships among healthcare providers as a contributing factor to successful NH resident transitions (Robinson et al., 2012). Since providers were ultimately responsible for decisions, trusted nurses were critical for ensuring the residents were well cared for. Nurses wanted providers to listen to them. They spoke favorably of providers who encouraged nurses to voice their opinions and felt disrespected by providers who ignored them, especially since hospitalization decisions impacted resident care and nurse workflow. Other research suggests that providers’ communication practices (e.g., using humor or empathic messages) could be a strong predictor of nurses’ satisfaction with “job, communication, relationships, and collaborative medical practices.” (Wanzer et al., 2009) However, the content of these discussions should be examined and clarified. SBAR is one way that nurses and providers suggested communication could be facilitated, though it is not used consistently. Some participants suggested that nurses might informally follow the steps of SBAR, without following the acronym exactly. In another study, almost 90 % of nurse respondents evaluated SBAR as useful for communicating with providers (Renz et al., 2013). However, we heard that when providers did not listen, nurses might not fully utilize SBAR: “why go through the trouble of a full presentation of the case if the patient will be sent out for hospitalization anyway.” Overall,

our findings are consistent with studies that highlight the importance of concise and prepared nurse reports and providers being “more open to listening.” (Tjia et al., 2009)

4.3. Emotions

Nurses and providers discussed emotions differently, which could further impact communication. Nurses were often able to vocalize emotions: they expressed frustration when providers did not listen, pleasure when providers recognized their good suggestions, and relief when providers made the right call, even if it went against earlier recommendations. In contrast, providers had a harder time talking about feelings. When asked how they felt, providers focused on the facts of the case and how facts led to optimal decisions. When prompted, they would mention the anxiety inherent in offsite decisions and their relief when things turned out well for the patient. At the same time, they mentioned the importance of “gut” decisions. It must be recognized that nurses and providers employ a great deal of intuitive understanding when making decisions (Hams, 2000). Previous work has identified that nurses’ training emphasized emotions more than physicians’ training, which can breed conflicts in communication (Foronda et al., 2016). Addressing this could help improve interprofessional conversations.

Factors outside of the team could also impact decision-making, including culture, residents and families, structures, and external groups.

4.4. Culture

NH cultures impacted nurse-provider relationships. A facility’s culture could emphasize opportunities for nurse-provider collaboration and relationship-building by empowering nurses to participate in decisions and expecting them to ask relevant questions before contacting providers. Another facility’s culture could minimize providers’ onsite presence or leave them to make decisions without the nurses’ input. While we emphasize the value of interprofessional relationships, we understand that a facility’s culture can shape the relationship quality of nurses and providers, as well as patient safety and quality of care (Boyle & Kochinda, 2004).

4.5. Residents and families

All participants mentioned how families impact hospitalization decisions. Families give critical information aiding decision making as they work with providers to ensure optimal outcomes for patients (Papautsky et al., 2017). Nurses described how families could push them to hospitalize, trumping the providers’ and nurses’ recommendations. Providers described family as powerful resident advocates. However, residents’ preferences ultimately guided providers’ decisions, “if a resident were ready to stop fighting and asked not to be sent to the hospital,” providers would honor their wishes and respect the patients’ autonomy even if that conflicted with their medical judgement or family wishes.

4.6. Structure/environment

Consistent with previous literature, participants highlighted the role of structural/environmental factors in hospitalization decisions, including the impact of “quantity and type of NH staff” on preventable hospitalizations (Grabowski & O’Malley, 2014; Gruneir et al., 2010). Suggestions included: better equipment for monitoring residents (e.g., onsite EKGs) and more manageable workloads with proper staffing, so that nurses had sufficient time to actually use said equipment. As noted elsewhere, NHs should develop facility resources (e.g., diagnostic equipment, staffing, training) to provide time sensitive and high-quality care to residents (Trahan et al., 2016). Consistent on-site leadership also seemed important for supporting nurses and improving their ability to build trust with providers (Begat et al., 2005; Britton et al., 2017; Clark

et al., 2017). Sites with high leadership turnover may have worse nurse-provider relationships.

4.7. External groups

Finally, previous studies show that “communication gaps in transfer decisions led to preventable hospital transfers.” (Lamb et al., 2011) and “poor quality discharge communication from the hospital hinders patient transitions and impacts the quality of care in NHs” (King et al., 2013; Lamb et al., 2011) supporting our findings. The lack of interoperability between hospitals and NHs complicates care and increases the risk of future hospitalizations.

In terms of practice, our findings suggest the need for nursing homes to invest in time and space for providers to get to know nurses professionally. This might be through lunch and learn sessions, or it might be through requests for interprofessional development during on-site visits. It could also be useful to provide training to foster shared knowledge and mutual respect, including tools like the SBAR, for organizing information and reporting it during team visits. Finally, it will be useful for organizations to emphasize shared goals for resident care and to protect their employees.

5. Limitations

Our study has some limitations that are important to note. First, this study was conducted with two Chicagoland nursing facilities, so its findings cannot be generalized. Second, it would have been helpful to include other entities involved in the hospitalization decision-making processes, such as residents and their families. Additional limitations include the difficulties in scheduling and implementing the focus groups and interviews due to the nature of the NH work environment. At the same time, this study provides some initial insight into the challenges facing nurses and providers as they engage in interconnected processes to assess patient hospitalization needs.

6. Conclusion

Preventable hospitalizations of NH residents may be reduced if more attention is paid to interpersonal factors of the care team, including perceived competence, respect, and trust, together with structural, environmental, and cultural factors.

Ethical approval

This study was approved by the University of Illinois Chicago Institutional Review Board.

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We certify that this work is novel or confirmatory of recent novel clinical research.

CRediT authorship contribution statement

All the authors have contributed to this manuscript through data collection, data analysis, writing, and revision. G.L., M.K., and E.S. conceptualized and designed the study. G.L., M.K., R.M., and E.S. collected data. G.L., M.K., R.M., P.P., E.K., S.P., and E.S. analyzed the data. G.L., M.K., U.A., C.G., and E.S. wrote and reviewed the manuscript. E.S. provided main supervision.

Declaration of competing interest

No conflict of interest.

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